

Policy on Transfusion Services

«Hospital Name»

Document control

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Prepared by	Blood Transfusion Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Blood and blood components are collected, tested, stored, and given to patients safely, with rational use and complete pre-transfusion verification every time.

1. Purpose

This policy explains how «Hospital Name» keeps its blood supply available and safe, from donor collection through to giving blood to a patient, verifies the correct patient and correct blood before every transfusion, tracks and responds to transfusion reactions, and runs a rational-use committee for blood.

This policy does not cover general informed consent procedures or infection control in detail — those are covered in other hospital policies.

2. Scope

This policy applies to blood centre staff, transfusion services staff, and clinical staff involved in ordering, administering, or monitoring blood transfusions at «Hospital Name».

3. Policy standards

«Hospital Name» keeps blood and blood components available from a registered blood centre — in-house or outsourced — collects, tests, stores, and distributes them under written guidance, ensures safe and rational use through a Hospital Blood Transfusion Committee, defines how quickly blood must be available in an emergency, and tracks and responds to transfusion reactions.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not use an unregistered blood centre — blood and blood components come only from a registered in-house or outsourced blood centre, with an MoU if outsourced, and patient care must not suffer for want of blood or components.
- Do not release blood before all required pre-transfusion tests, including Transfusion Transmissible Infections (TTI) testing and Red Cell Serology, are complete using approved methods.
- Do not take a quarantined unit into inventory before re-testing is complete.
- Do not mix untested and tested blood units in storage, and do not label a unit's serology status before TTI screening reports are released.
- Do not start a transfusion without completing all eight elements of the written guidance: patient consent for blood administration, safe procurement and transportation maintaining the cold chain, patient identification, verification of the blood administration order, safe storage prior to transfusion, blood administration, monitoring of the patient, and identification of and response to potential transfusion reactions.
- Do not start transfusion of blood or blood components when patient identification (minimum two identifiers) is incomplete, compatibility checks are missing, or required informed consent has not been obtained, except where the organisation's documented emergency-transfusion guidance applies.
- Do not leave the emergency-use time frame for blood availability undefined, or leave routine and emergency turnaround times undefined or unmonitored.
- Do not skip detection, reporting, evaluation, or analysis of a suspected adverse transfusion reaction, and do not skip corrective and preventive action based on that analysis.
- Do not operate without participating in the Haemovigilance Programme of India.

5. What we do

5.1 Keep blood available from a registered blood centre

Blood and blood components are available from either an in-house or an outsourced registered blood centre. The blood centre has adequate infrastructure, staff, and equipment for its workload and scope, so operations run smoothly without delays.

Where the blood centre is outsourced, there is a signed MoU, and patient care does not suffer for want of blood or components — blood is transported from the external blood centre safely and properly.

The NABH standards for blood centres are a useful reference.

5.2 Follow written guidance for blood centre activities

Written guidance covers nine blood centre activities: blood donor selection; blood screening for transfusion-transmissible diseases; blood collection, including apheresis procedures; blood storage; compatibility testing; blood distribution; transfusion of blood and products in clinical areas; discard of sero-positive or unutilised blood; and education of families of potential donors and patients regarding blood donation.

Blood and apheresis products are collected only from voluntary, non-remunerated, low-risk, safe, and healthy donors, with pre- and post-donation counselling and informed consent. Apheresis procedures follow relevant guidelines. Blood donation drives follow national and statutory guidelines. Drugs and equipment for treating donor reactions are available, and staff are trained to use them. Adverse donor reactions are identified, managed, and reported in line with the National Haemovigilance Programme of India.

All pre-transfusion tests — including Transfusion Transmissible Infections (TTI) testing and Red Cell Serology — are carried out using approved methods before blood is released. Recipient and donor blood samples are retained and stored after each transfusion, as per guidelines. If a quality-control failure occurs during TTI testing, quarantined units are not taken into inventory until re-testing is complete. There is a process for retesting, recall, and referral of reactive blood donors, in line with national guidelines. Whole blood or components from any unit that tests positive or reactive are discarded as per written guidance.

5.3 Store blood safely from collection to transfusion

The blood centre prepares components according to its scope. The temperature of whole blood, its components, and the storage environment is maintained and recorded as per guidelines.

Untested blood and products are quarantined, with separate, designated storage for untested and tested units based on serology status. Labelling — including serology status — happens only after TTI screening reports are released, so that untested and tested blood cannot get mixed up.

5.4 Ensure safe and rational transfusion

Transfusion of blood and blood components follows written guidance covering eight things: patient consent for blood administration; safe procurement and transportation, maintaining the cold chain and correct source; patient identification; verification of blood administration orders; safe storage prior to transfusion; blood administration; monitoring of the patient; and identification of and response to potential transfusion reactions.

Measures are in place to prevent mismatched transfusion under all circumstances — for example, confirming patient identity at cross-matching, attaching the compatibility testing label to the unit at issue, confirming patient identity and blood group again at the time of transfusion, keeping a portion of the integral tubing with its segment number attached to the blood bag at issue, and keeping all labels attached to the blood bag until the transfusion

is complete, so any adverse reaction can be properly investigated. These are examples of how the prevention principle is put into practice, not a fixed checklist.

Written guidance also governs the indications for using blood and blood components, based on standard practice guidelines or sound clinical practice from national and international professional bodies, and covers inventory and ordering schedules, both planned and unplanned. Protocols could address specific clinical situations relevant to the hospital's scope — for example, neonatal transfusions, autoimmune haemolytic anaemia, incompatible transfusion, or massive transfusion.

A Hospital Blood Transfusion Committee (HBTC) ensures the rational use of blood by adopting or adapting rational-use guidelines. It is preferably chaired by a clinician, and includes representation from clinical departments, the blood transfusion officer, nursing, the Quality Coordinator, and management. The committee meets periodically and monitors the availability of blood and components, transfusion practices and audits, adverse transfusion reactions, and wastage of blood or products. The committee could also seek periodic feedback from clinicians on the availability and effectiveness of blood and components.

5.5 Define how fast blood must be available

«Hospital Name» defines what counts as an emergency use of blood — covering both actual and anticipated need — and puts procedures in place to make sure blood is available for it.

The time frame within which blood must be available in an emergency is defined by «Hospital Name» and documented in the Blood Centre SOP maintained by the Blood Transfusion Officer. Turnaround time for both routine and emergency blood issues is defined and monitored. This applies even if the hospital does not have its own in-house blood centre.

5.6 Track and respond to transfusion reactions

Suspected adverse transfusion reactions are detected, reported, evaluated, and analysed, with corrective and preventive action taken afterward. It is preferable to capture feedback on every transfusion, so no reaction goes unnoticed — analysis can be done by an individual or a committee, as «Hospital Name» decides. A record of transfusion reactions is maintained.

«Hospital Name» participates in the Haemovigilance Programme of India. (Related requirements are covered in the hospital's other policies.)

5.7 Run a quality assurance programme for the blood centre

The quality assurance programme — whether standalone or part of the hospital's overall quality improvement programme — covers every aspect of the blood centre's work.

The blood centre takes part in an External Quality Assurance Scheme (EQAS) or Proficiency Testing (PT) programme, or a suitable interlaboratory comparison if EQAS/PT is not

available. Results from internal quality control, EQAS, PT, or interlaboratory testing are monitored, with corrective and preventive action taken and records kept. Blood, products, and reagents are sampled at a defined frequency, as specified in the Blood Centre Quality Management SOP maintained by the Blood Transfusion Officer, for internal quality control, following guidelines.

The blood centre collects data on key performance indicators as part of its quality improvement work — for example, wastage of blood and products, or the rate of transfusion reactions. This data is collated, analysed, and used to drive further improvements, which are monitored to make sure they hold.

6. Stop-work authority

Do not start transfusion of blood or blood components when patient identification (minimum two identifiers) is incomplete, compatibility checks are missing, or required informed consent has not been obtained, except where the organisation's documented emergency-transfusion guidance applies.

Stop-work applies to the transfusion start. Life-saving emergency transfusion follows the organisation's emergency-transfusion written guidance and is documented.

The person who stops tells the Transfusion / Blood Bank In-Charge and the treating doctor the same shift. Refusing an unsafe transfusion is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and appropriately resourced.
Blood Transfusion Officer	Oversees blood centre operations, written guidance for all nine activities, pre-transfusion testing, storage, and quality assurance; receives stop-work notifications the same shift.
Hospital Blood Transfusion Committee (HBTC)	Ensures rational use of blood; preferably chaired by a clinician; includes representation from clinical departments, the blood transfusion officer, nursing, the Quality Coordinator, and management; meets periodically.
Clinical staff (ordering and administering transfusions)	Follow all eight transfusion written-guidance elements; complete pre-transfusion verification; monitor patients; report suspected adverse reactions.
Blood centre staff	Follow written guidance for all nine blood centre activities; maintain storage and quarantine requirements; carry out pre-

	transfusion testing using approved methods.
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8. Quality monitoring

Monitoring area	What is monitored
Registered blood centre	Use of a registered blood centre; MoU in place where outsourced
Pre-transfusion testing	TTI testing and Red Cell Serology complete using approved methods before blood is released
Storage and quarantine	Separate storage of untested and tested units; labelling only after TTI reports released
Transfusion verification	All eight written-guidance elements completed before every transfusion
Blood availability	Defined emergency and routine turnaround times; monitoring against the organisation's defined time frames
Adverse transfusion reactions	Detection, reporting, evaluation, analysis, and CAPA for suspected adverse reactions; record of transfusion reactions maintained
Haemovigilance	Active participation in the Haemovigilance Programme of India
Quality assurance programme	EQAS/PT participation; internal quality control at defined frequency; KPI data collected, analysed, and used for improvement

9. Training and staff acknowledgement

Blood centre staff and clinical staff involved in transfusion shall be familiar with the written guidance applicable to their work, including pre-transfusion testing, storage requirements, the eight-element transfusion process, adverse reaction reporting, and Haemovigilance Programme requirements.

I have read the Policy on Transfusion Services of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to blood centre staff, transfusion services staff, and clinical staff involved in ordering, administering, or monitoring blood transfusions.

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
EQAS	External Quality Assurance Scheme
HBTC	Hospital Blood Transfusion Committee
MoU	Memorandum of Understanding
PT	Proficiency Testing
TTI	Transfusion Transmissible Infections

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.8.a	Commitment	Section 5.1 addresses availability of blood and blood components from a registered in-house or outsourced blood centre, with an MoU where outsourced, and adequate infrastructure, staff, and equipment.
COP.8.b	Commitment*	Section 5.2 addresses written guidance for all nine blood centre activities: blood donor selection; TTI screening; blood collection including apheresis; blood storage; compatibility testing; blood distribution; transfusion in clinical areas; discard of sero-positive or unutilised blood; and donor/patient family education.
COP.8.c	Commitment	Section 5.3 addresses safe storage from collection to transfusion: component preparation, temperature maintenance and recording, quarantine of untested

		units, separate designated storage by serology status, and labelling only after TTI reports are released.
COP.8.d	CORE*	Section 5.4 addresses written guidance covering all eight transfusion elements: patient consent; safe procurement and transportation; patient identification; verification of blood administration orders; safe storage prior to transfusion; blood administration; patient monitoring; and identification of and response to potential transfusion reactions. Also addresses mismatch prevention measures, rational-use indications, and the HBTC. Stop-work authority (Section 6) also applies to this element.
COP.8.e	Commitment*	Section 5.5 addresses the organisation-defined emergency blood availability time frame and defined, monitored turnaround times for routine and emergency blood issues.
COP.8.f	Achievement*	Section 5.6 addresses detection, reporting, evaluation, analysis, and CAPA for suspected adverse transfusion reactions; maintenance of a transfusion reaction record; and mandatory participation in the Haemovigilance Programme of India.
COP.8.g	Achievement*	Section 5.7 addresses the quality assurance programme covering all blood centre activities, EQAS/PT participation or interlaboratory comparison,

		internal quality control at a defined frequency, and KPI data collection, analysis, and improvement monitoring.
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13. Required Records/Evidence Checklist

Registered blood centre and outsourcing

- Evidence that blood and blood components come from a registered in-house or outsourced blood centre.
- Signed MoU with the outsourced blood centre where applicable.
- Evidence that patient care does not suffer for want of blood or components.

Written guidance — nine blood centre activities

- Written guidance for blood donor selection.
- Written guidance for blood screening for transfusion-transmissible diseases.
- Written guidance for blood collection, including apheresis procedures.
- Written guidance for blood storage.
- Written guidance for compatibility testing.
- Written guidance for blood distribution.
- Written guidance for transfusion of blood and products in clinical areas.
- Written guidance for discard of sero-positive or unutilised blood.
- Written guidance for education of families of potential donors and patients regarding blood donation.

Donor selection and blood collection

- Records showing blood and apheresis products collected only from voluntary, non-remunerated, low-risk, safe, and healthy donors.
- Evidence of pre- and post-donation counselling and informed consent.
- Drugs and equipment for treating donor reactions available; staff trained to use them.
- Adverse donor reaction records reported in line with the National Haemovigilance Programme of India.

Pre-transfusion testing and quarantine

- Records showing TTI testing and Red Cell Serology carried out using approved methods before blood is released.
- Records of recipient and donor blood sample retention and storage after each transfusion.
- Evidence that quarantined units are not taken into inventory until re-testing is complete after a quality-control failure.
- Process records for retesting, recall, and referral of reactive blood donors in line with national guidelines.

- Records showing whole blood or components from reactive units discarded as per written guidance.

Storage

- Temperature records for whole blood, components, and the storage environment.
- Evidence of separate, designated storage for untested and tested units by serology status.
- Records showing labelling — including serology status — done only after TTI screening reports are released.

Transfusion — eight written-guidance elements

- Patient consent for blood administration.
- Records showing safe procurement and transportation maintaining the cold chain and correct source.
- Patient identification completed before transfusion.
- Verification of blood administration orders.
- Safe storage prior to transfusion.
- Blood administration records.
- Patient monitoring records during and after transfusion.
- Records of identification of and response to potential transfusion reactions.

Mismatch prevention and rational use

- Evidence of mismatch-prevention measures in place under all circumstances.
- Written guidance on indications for blood and blood components based on standard practice guidelines or sound clinical practice.
- Inventory and ordering schedules, both planned and unplanned.

Hospital Blood Transfusion Committee

- Evidence that an HBTC is in place, preferably chaired by a clinician, with representation from clinical departments, the blood transfusion officer, nursing, the Quality Coordinator, and management.
- Records of periodic HBTC meetings monitoring blood availability, transfusion practices and audits, adverse reactions, and blood/product wastage.

Emergency blood availability

- Documented definition of emergency use of blood, covering actual and anticipated need.
- Defined emergency blood availability time frame (set by «Hospital Name»), with evidence of monitoring against it.
- Defined and monitored turnaround times for both routine and emergency blood issues.

Adverse reactions and Haemovigilance

- Records showing suspected adverse transfusion reactions detected, reported, evaluated, and analysed.

- CAPA records for adverse transfusion reactions.
- Record of transfusion reactions maintained.
- Evidence of active participation in the Haemovigilance Programme of India.

Quality assurance programme

- Quality assurance programme covering all blood centre activities — standalone or part of the overall quality improvement programme.
- Evidence of EQAS or PT participation, or interlaboratory comparison where EQAS/PT is not available.
- Records showing internal quality control, EQAS/PT/interlaboratory results monitored, with CAPA and records kept.
- Evidence of blood, product, and reagent sampling at a defined frequency for internal quality control.
- KPI data (for example, blood/product wastage, transfusion reaction rates) collected, analysed, and used for improvement; improvements monitored.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.8.
2. Guidebook interpretation supplied for COP.8.a through COP.8.g.
3. National Haemovigilance Programme of India guidelines.
4. NABH Standards for Blood Centres (reference for blood centre requirements).

Disclaimer

This policy reorganises the supplied COP.8 objective elements and approved plain-language content into policy format. The stop-work text is reproduced exactly as supplied. Mandatory requirements and their stated modal strength have been retained. The nine COP.8.b blood-centre written-guidance activities, the eight COP.8.d transfusion written-guidance elements, the HBTC mandatory composition, mandatory Haemovigilance Programme participation, and the minimum-two-identifiers stop-work quantity have been preserved. No numerical time frame for emergency blood availability has been invented; the time frame is defined by the organisation.